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To: Department of Health and Human Services (HHS), Office of the National Coordinator (ONC)

Re: RFI: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (Docket No. 2025-23641)

From: xCures, Inc.

Executive Summary

xCures operates an AI-assisted healthcare data platform that extracts and structures clinical information from medical records gathered across multiple sources, including from health information networks like TEFCA and Carequality. This platform serves as a semantic layer for healthcare data, delivering decision-ready intelligence that empowers professionals to act with confidence and clarity.

We appreciate the opportunity to comment on how HHS can accelerate AI adoption in clinical care. Our experience processing millions of patient records across the U.S. healthcare system suggests that the primary barrier to AI efficacy is not algorithm availability, but data readiness—specifically, the inability of standard systems to process unstructured clinical evidence (PDFs, faxes, and images) where the most critical medical insights reside.

We respectfully submit the following responses to specific questions in the RFI, advocating for a regulatory framework that prioritizes Source Verifiability (audit trails), Universal Participation, and Semantic Interoperability.

Responses to Specific Questions

1. What are the biggest barriers to private sector innovation in AI for health care?

Barrier A: The "Unstructured Data" Cliff

The greatest technical barrier is the industry's over-reliance on structured data feeds (HL7/FHIR) which often fail to capture the complete clinical picture.

- The Reality: While a majority of health data transmission may technically be structured (XML/JSON), in reality the high-value clinical indicators—pathology reports, genomic staging, and discharge summaries—predominantly reside in unstructured formats like PDFs, faxes, and scanned images.
- The Barrier: Current "standard" interoperability often dumps these files as raw blobs that generic AI models struggle to interpret accurately. Without an infrastructure layer capable of normalizing this unstructured multimedia data into compute-ready formats, clinical AI will remain limited to basic administrative tasks rather than deep clinical decision support.

Barrier B: Data Access and Network Participation

Even when data is digital, another barrier is just getting access to all the data. Not all holders of patient's health data are participating in TEFCA.

- **The Barrier:** This non-universal participation in health information networks means AI models cannot build a complete, longitudinal view of the patient, leading to "hallucinations" born of missing context.
- **The Solution:** Innovation requires a federal requirement or penalty for not participating in TEFCA's treatment and individual access services pathway, which are critical for healthcare providers and patients to have access to their longitudinal data in order to use that for AI. Streamlining access through mandatory participation and seamless access would allow healthcare providers and patients to rapidly aggregate the necessary data to power clinical AI, reducing the cost and time delays currently imposed by fragmented retrieval processes.

2. What regulatory or payment policy changes should HHS prioritize?

Incentivize "Data Completeness" and Administrative Reduction:

HHS should prioritize payment models that reimburse for the completeness of clinical documentation, not just the volume of care.

- **Reimbursement Leakage:** Healthcare organizations currently face significant revenue loss due to missing or incomplete documentation. This "administrative friction" delays patient care (e.g., prior authorization bottlenecks).
- **Recommendation:** CMS should allow AI-verified data (with direct links to source documents) to automatically satisfy documentation requirements for Prior Authorization and Quality Reporting (e.g., NCQA ECDS). This would incentivize the adoption of AI tools that solve the "missing data" problem, directly reducing administrative burden.

4. & 5. What are the most promising AI evaluation methods? How can HHS support certification?

The "Trust but Verify" Standard:

HHS must distinguish between "Black Box" AI (which summarizes without proof) and Source-Verified AI.

- **Safety Risk:** Generative AI that summarizes patient history without an audit trail poses a significant safety risk (hallucinations).
- **Proposed Standard:** We advocate that any AI tool used for clinical decision-making or reimbursement must meet a "Source Linking" standard. Every extracted clinical fact (e.g., "Patient has Stage 4 Breast Cancer") must be digitally linked to the specific page, document, and timestamp of the source record.
- **Validation:** Validation metrics should not just measure "accuracy" against a gold standard, but the *speed and ability* of a human reviewer to verify the AI's claim via these source links. This "human-in-the-loop" architecture is the only safe path for clinical AI.
- **Ongoing Quality Control:** Initial validation is insufficient given the rapid evolution of AI technology. We recommend regulatory frameworks mandate ongoing quality controls—such as

continuous monitoring and regression testing—to ensure that AI tools remain trustworthy and stable as models and data sources evolve.

8. Where would enhanced interoperability widen market opportunities?

A. Mandate Universal Network Participation

To widen market opportunities for AI, interoperability must be universal, not optional.

- **The Problem:** Currently, participation in networks like TEFCA is voluntary, leaving significant gaps in coverage—particularly among smaller practices, rural clinics, and specialists who may still rely on fax. AI models trained on or operating with "Swiss cheese" data (data with holes) are inherently unsafe and biased.
- **Recommendation:** HHS should prioritize regulatory levers that move towards mandatory participation in Health Information Networks for all providers. Universal participation ensures that AI agents have access to the *entire* patient journey, not just the episodes that occurred in large, tech-enabled health systems.

B. From "Transport" to "Semantic" Interoperability

Current interoperability frameworks (TEFCA, QHINs) excel at transporting files but fail at semantic understanding.

- **The Gap:** Moving a PDF from Hospital A to Hospital B is "interoperability," but it does not help an AI system understand the patient's history.
- **Evidence:** In representative analyses of patients with complex conditions (e.g., advanced oncology), we found that relying solely on standard structured data feeds resulted in missing approximately 88% of relevant Hierarchical Condition Category (HCC) coding opportunities compared to AI-enhanced extraction from unstructured notes.
- **Recommendation:** HHS should broaden the definition of "interoperability" to include the capacity to parse, normalize, and structure unstructured data. True interoperability is not just moving the file; it is making the *content* of the file machine-readable for downstream AI applications.